

Petscop: The game that never existed

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Deeparghya Dutta Barua



With tons of content relating to conspiracy theories meandering through YouTube's algorithm, one would be tired from binging through all the questionable and sometimes dubious stuff that float around your suggested videos. Petscop uses the same curiosity enticing formula to hold the viewer's attention. However, it's not a poorly executed series based off of the same 10 things that actually never happen in the deep web, but rather relies on the same sentiment found in creepypastas, albeit with its own distinct touch.

Petscop introduces itself as a game developed by a studio named Garalina, none of which is true because it's actually a web series following the "Let's Play" (LP) format. It's speculated that the game is real since it would take much more effort to make the content as videos instead of using a game engine. The intriguing fact here is that the game itself was created solely for the purpose of making videos in the LP format, not the other way around as it should be with any other gameplay video. The person recording the gameplay identifies himself as Paul, who seems to hold connections with the story of the game. He plays a genuinely confused role throughout the entire web series and what makes it compelling is how natural and unscripted it feels.



The game starts off with bright and cheery themes found in console-exclusive puzzlers from the 2000s. The game design that worked behind the making of the videos makes it obvious that someone well versed in game development had his hand over the entire process. Although the descriptions read throughout the "Even Care" portion of the gameplay seem light-hearted, they allude themselves in later portions of the game as foreshadows of much more difficult issues. Every single dialogue in the game indicates something and their initial incoherence or ambiguity is usually dispelled with the progression of the game.

The second and much larger portion of the game, located in the "Newmaker Plane", can be unlocked via a special combo of buttons. The eerie nature of the flipped side is where the story, the metaphors and the allusions start arching on top of each other, creating quite a confusing experience. Although there are very few jump scares, the lack of background music and narration and in some cases, looped or glitch sound effects add to the unsettling feeling. In fact, the execution is so well done that it explicitly doesn't need to resort to typical horror tropes to portray itself as something intimidating.

Although the game carries lots of external references as well, the most evident resemblance with any real life instances would be the infamous Candace Newmaker case, which is brought up over and over through the concepts of rebirthing, adoptive parenting and direct references to names such as the "Newmaker Plane" or the character, Tiara.

What I personally like about the whole thing is its fresh approach to a dated internet trend, the complex storytelling format, and the usage of deception to describe the game and also for solving puzzles within the game itself. The game deconstructs its cutesy façade to embrace something with a much more grim and dark undertone. As the web series is still in progress, the only way to have an interactive experience for the moment being would be 'Giftscop', a fan-made rendition which receives regular updates.



Deeparghya Dutta Barua likes to feel apprehensive whenever there are more than two people around. Help him in finding new ways of butchering his name at arg274@pm.me



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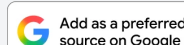
Dr Tajul Islam A Bari
Public Health Specialist & Vaccinologist



Dr Tajul Islam A Bari, a public health specialist and vaccinologist, who previously served as manager of the national immunisation programme, speaks with **Naznin Tithi of The Daily Star** about the recent measles outbreak, immunisation gaps, and measures required to eradicate measles from the country.

There has never been such a high number of child deaths from measles in Bangladesh in the last couple of decades because of the success of our Expanded Programme on Immunization (EPI). How would you evaluate the government’s response to this outbreak?

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The measles vaccine is administered in two doses—one at nine months and the other at 15 months. When at least 95 percent of children in each district of the country receive both doses, transmission of the virus can be stopped. However, a significant immunity gap, as indicated by the coverage evaluation survey, has led to the current outbreak. Bangladesh is a densely populated country with relatively high rates of malnutrition, making children more vulnerable.





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Measles Vaccination: Govt fast-tracks nationwide drive

During the tenure of the interim government, Tk 458 crore was released on December 28, 2024, for the procurement of vaccines. The government also allocated Tk 1,451 crore for the next fiscal year (2025–26). However, due to lack of approval from the relevant authorities, the funds could not be released. As a result, no vaccines were purchased in 2025. However, the current government has taken steps to release that fund for vaccine procurement. The vaccines are expected to arrive soon for our EPI.

The interim government initially stated that half of the vaccines would be procured by the government and half by Unicef. However, the current government has decided that all vaccines will now be procured through Unicef.

By what year was measles supposed to be eliminated in Bangladesh? How far had we progressed towards that goal before the outbreak?



Before eradication, there are several stages. Bangladesh, along with several other South Asian countries, had planned to eliminate measles by 2026. This means reducing cases to zero per one million population. In 2024–25, Bangladesh had achieved a rate of 0.72 per million, which is close to the threshold level. In 2025, the target was to bring it down to zero—meaning no cases per million population. This is called elimination. If achieved by 2025, elimination certification could have been obtained in 2026.

To eliminate measles, large-scale supplementary campaigns must be conducted periodically to ensure broader protection. In 2005, such a campaign was conducted in one city corporation and two districts. In 2006, it was expanded to the remaining city corporations and 62 districts. One of the largest measles campaigns in the world was conducted from January 25, 2014, to February 13, 2014, vaccinating more than 50 million children aged nine months to under 15 years. This is known as the national MR catch-up campaign. Following this, measles incidence declined significantly. The standard practice is to conduct such campaigns every 4–5 years to address immunity gaps. The last follow-up campaign took place in December 2020. All measles campaign costs were supported by Gavi, the Vaccine Alliance, covering both vaccine procurement and operational expenses.

Later, the government planned another campaign, which was scheduled for 2024. An application was submitted to Gavi, and it was approved after review. The decision letter confirming support was sent to Bangladesh on March 7, 2025. Gavi provided around Tk 600 crore to vaccinate 2.1 crore children, along with USD 17.8 million for operational costs.





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Measles outbreak in year of elimination target

Although the campaign was supposed to be conducted in December 2025 or January 2026, it was delayed due to elections and other reasons. The same vaccine stock is now being used for the outbreak response campaign launched on April 5, 2026, in 30 high-risk upazilas.

Apart from the emergency vaccination programme, the government plans further campaigns. Are such initiatives sufficient?

The current campaign will be implemented in three phases. In the first phase, 30 upazilas and high-risk areas across 18 districts will be covered. The second phase will begin on April 12 in various areas of Dhaka city, as well as in Barishal and Mymensingh. From May 3, a nationwide campaign will begin for children aged six months to under 10 years.

Measles is a highly infectious disease and 95 percent coverage is required at the district level, which has not yet been accomplished. The latest coverage [evaluation survey](#) (2023) shows that first-dose coverage is around 86.1 percent, while second-dose



coverage is around 80.7 percent. As a result, the number of unvaccinated children continues to grow. Bangladesh has around 34.1 lakh births per year, but not all children are being vaccinated.

No vaccine in the world is 100 percent effective. The vaccine given at six months is about 50 percent effective, at nine months, about 85 percent, and at 15 months, more than 90 percent. Therefore, even after vaccination, full immunity is not always achieved. Those who are unvaccinated are at even greater risk. Almost half a million children remain susceptible each year due to non-vaccination or failure to develop immunity after vaccination. When the number of unvaccinated or under-vaccinated children accumulates over the years, outbreaks of this type occur.

The ideal strategy is nationwide mass vaccination campaigns to ensure broader protection and ultimately close the immunity gap. This involves going door to door to identify eligible children and vaccinating them. Whether or not a child has previously been vaccinated, they should receive at least one dose in this campaign. However, there is a condition: the vaccine used in this campaign cannot be administered within one month of the last dose of the MR vaccine.

How can our immunisation programme be strengthened further so that no child has to die from preventable diseases?

To strengthen Bangladesh's immunisation programme, vaccines need to be produced by local public or private manufacturers and must obtain prequalification from the World Health Organization (WHO) Headquarters in Geneva. Without WHO's prequalification of the vaccine, Gavi support cannot be accessed. There are specific requirements for prequalification. But for that, the National Control Laboratory (NCL), under the Directorate General of Drug Administration, needs to be strengthened. EPI vaccines are usually procured from foreign countries, with Unicef acting as the procurement agency. A large amount of money flows abroad for the procurement of these vaccines, which could potentially be purchased domestically if they are WHO-prequalified. Urgent attention should be given to this issue.

What steps should the government take to prevent a similar outbreak in the future?



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To prevent such situations in the future, surveillance must be strengthened. When measles cases occur, blood samples must be collected and laboratory results must be properly shared with the programme personnel. Wherever a case is detected,



vaccination must be conducted in and around the affected area for unvaccinated individuals. Even those who have been vaccinated may require a repeat dose if more than one month has passed since their last dose.

Gaps in routine immunisation—especially at nine months and 15 months—must be addressed. It must be ensured that at least 95 percent of children in every district receive both doses and that this coverage is maintained annually. Door-to-door child identification, community engagement, and awareness-raising efforts must be strengthened. Birth registration must be properly done so that every child can be vaccinated on schedule. These tasks are carried out by health assistants and family welfare workers. But a large number of these posts in multiple districts remain vacant. These vacancies must be filled urgently, requiring coordination between the Ministry of Health and the Ministry of Finance. Porters responsible for transporting vaccines from upazila health complexes to distribution points, from where field workers collect vaccines for administration at vaccination sites, are also employed within the immunisation system. Their salaries were previously paid from the government's development budget or from Gavi-supported Health System Strengthening (HSS) funds. However, for the last several months, their salaries have remained unpaid. That payment must also be ensured.

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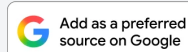


With hope and aspiration for a better tomorrow, we welcome Bangladesh's new prime minister, Tarique Rahman, his cabinet members, and the members of the 13th parliament from both the ruling coalition and the opposition. The oath-taking ceremony of the new legislative and executive bodies on Tuesday marked the

beginning of our country's transition back to parliamentary democracy. Empowered by ballots, they now bear the responsibility of shaping the nation's future for, hopefully, the next five years.

The greater responsibility, however, rests with the Bangladesh Nationalist Party (BNP), which formed the government with a two-thirds majority. Given our less-than-encouraging experience with majoritarian governments, particularly in the recent past, the new ruling party must take care not to misuse that power. Encouragingly, Tarique Rahman's measured words and gestures following the election results signalled a positive shift. So far, both the victors and the opposition alliance have shown relative restraint in their post-election reactions, something that has often been absent during previous transitions.

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BNP's decision to refrain from taking the oath as members of the Constitution Reform Council may create some confusion. Under the July National Charter (Constitution Reform) Implementation Order, 2025, members of the 13th parliament are also to serve on the Constitution Reform Council. While several independent MPs, along with lawmakers from Jamaat-e-Islami, the National Citizen Party, and Islami Andolon, took the oath as council members on Tuesday, BNP MPs and independent Rumeen Farhana did not. BNP leader Salahuddin Ahmed told reporters that once parliament constitutionally approves the council, rules regarding members' swearing-in can be framed. He further argued that, under the existing constitution, lawmakers were not elected specifically as members of the Constitution Reform Council. While this justification merits consideration, as the ruling party the BNP must make sincere efforts to bridge differences with the opposition on this issue through dialogue. All the parties must remain resolute in their commitment to the principles and spirit of the July Charter, which they have all signed, the BNP included, who placed notes of dissent regarding some clauses.

Aside from the minor hiccup regarding the Constituent Reform Council, the new government's start has been promising. The decision by BNP and Jamaat-e-Islami lawmakers to forgo duty-free vehicles and government-allotted residential plots is commendable. At a time of inflationary pressure, such restraint could translate into meaningful national savings. We hope this spirit carries over into broader public expenditures as the executive assumes office.



As we watched emerging young leaders and seasoned stalwarts pledge to make our voices heard in the halls of democracy—the Jatiya Sangsad—we were reminded of the thousands whose sacrifices led to the ouster of an authoritarian regime during the July 2024 uprising. The 18-month interim administration that followed, despite its failure to implement many promised reforms, did deliver a credible election, and for that it deserves acknowledgment. Now, both the ruling party and the opposition must ensure that this hard-won democratic renewal does not falter again.



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